

GENERALISED ITCHY RASH

Check if the patient needs urgent attention ↗ 67.

If red itchy crops of bumps that may have blistered or healed with darkening of skin, may have scratch marks, **insects bites** likely →70.

Small red bumps and burrows in webspaces of fingers, axillae, waist and genitals. Very itchy, especially at night.



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Scabies likely

- Apply **benzyl benzoate 25%** lotion from neck to soles of feet and rub in well:
 - Leave on for 24 hours, then wash off with soap and water.
 - If severe, repeat once after 24 hours or within 5 days.
- Only if no better, apply **permethrin 5%** lotion at night from neck to soles of feet. Wash off after 8-12 hours. Repeat after 1 week if needed. Avoid using permethrin and benzyl benzoate together as may be toxic.
- For itch, give **chlorphenamine** 4mg 8 hourly for up to 10 days. If mild itch, use only at night.
- Advise can return to work after first treatment.
- Treat all household contacts and sexual partners at the same time, even if asymptomatic.
- Wash recently used linen and clothing in very hot water and dry well. Expose to direct sunlight.
- If yellow crusts, also treat for likely **impetigo** ↗ 78.

If no response to treatment, discuss/refer.

Hyperpigmented, itchy bumps on limbs, trunk or face



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Papular pruritic eruption (PPE) likely

- Test for HIV ↗ 110.
- If lesions in webspaces, axillae or genitals, also treat for scabies in adjacent column.
- Apply **hydrocortisone 1%** cream twice a day for 7 days (apply sparingly to face, avoid eyes)
- For itch, give **certirizine** 10mg daily.
- Advise patient:
 - Reduce exposure to insect bites.
 - May be long-standing and skin often remains hyperpigmented.
 - May temporarily worsen after starting ART.

Patches of dry, scaly, itchy skin on wrists, ankles, inside elbows or behind knees.



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Eczema likely

- Advise that eczema is a chronic condition with episodes of acute exacerbations.
- Advise to avoid triggers such as soap, detergents, heat, fabrics that cause itch, overheating at night.
- If low mood, stress or anxiety ↗ 86.
- If severe, start at Step 3.
- **Step 1.** Wash with **aqueous cream (UEA)** instead of soap. Moisturise skin with **emulsifying ointment (UE)** twice a day and immediately after bathing.
- **Step 2.** If no better after 7 days or more severe eczema: apply **hydrocortisone 1%** cream twice a day for 7 days (apply sparingly to face, avoid eyes). If good response, reduce to once a day for 3 days, then stop.
- **Step 3.** If poor response to hydrocortisone or severe eczema, apply instead **betamethasone 0.1%** ointment once a day for 7 days (avoid face and neck). If good response, reduce to once a day for 3 days, then stop.
- For itch, give **certirizine** 10mg daily.
- If oozing, pus or yellow crusts, treat for infection: give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days. If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- Refer if: no better after 2 weeks, extensive involvement or painful pustules.

Very itchy, red, raised wheals that appear suddenly and usually disappear within 24 hours

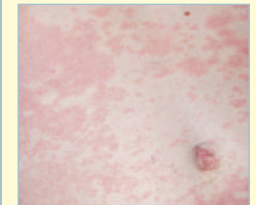


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Urticaria likely

- Help to identify and advise to avoid triggers².
- Apply **calamine lotion** as needed.
- If recurrent eye problem, exclude allergic conjunctivitis ↗ 31.
- If recurrent nose problem, exclude allergic rhinitis ↗ 34.
- If recurrent cough or wheeze, exclude asthma ↗ 123.
- For itch, give **chlorphenamine** 4mg 6-8 hourly.
- Advise to return immediately if any symptoms of anaphylaxis³ occur.
- If no better after 24 hours, refer.

Diffuse red rash mainly on trunk, arms and legs, which appeared within 3 months of starting a new medication.



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Drug reaction likely →73.

¹History of anaphylaxis, urticaria or angioedema. ²Common triggers include foods (milk, eggs, nuts, wheat, seafood), medications, insect bites/stings and latex. ³Symptoms of anaphylaxis include wheeze, difficulty breathing, dizziness/collapse, abdominal pain, vomiting.